

VYTALIX

Investor pitch

Pre-seed · September 2026 · Draft for discussion

THE PROBLEM

Maternity care fails most often between appointments, not during them

- UK reviews (Ockenden, Kirkup) repeatedly cite communication and coordination failures
- MBRRACE-UK shows persistent inequalities for Black and Asian women
- Nigeria carries one of the world's highest maternal mortality burdens, with weak continuity between women and facilities

Visual: Three source cards with citations; no stock photography.



WHO WE ARE

A group, not an app: services that fund the company and a product that defines it

- Advisory
- Consulting
- Health Solutions (MaternaLink, in development)
- E-Learning (year 2)

Visual: Four-pillar diagram with the flow "services cash → product build".



MATERNALINK (IN DEVELOPMENT)

A coordination layer between women, midwives and facilities

- Continuity of contact
- two-way messaging with human escalation
- clinician-governed education
- programme dashboards
- offline-tolerant, low-bandwidth, multilingual roadmap
- non-diagnostic by design

Visual: Simple journey line: antenatal → birth → postnatal with touchpoints.



WHY THE REGULATORY POSTURE MATTERS

Built to stay outside device classification, with a path if we ever cross the line

- Intended-use statement
- clinical safety officer and hazard log (DCB0129)
- UK GDPR, DPIA, data minimisation
- DTAC readiness
- AI risk-scoring kept as a research track behind ethics and a SaMD pathway

Visual: A line with "v1 coordination" on one side and "v3 research track" on the other.



Two entry markets with different buyers

- UK: NHS trusts and ICBs, private maternity, employers (services and pilots first)
- Nigeria: state programmes, NGOs, insurers, telecom distribution (design to complement existing programmes)
- Sizing in `/docs/05\_MARKET\_STRATEGY.md`, all ESTIMATE

Visual: Two columns; scored market table excerpt.



BUSINESS MODEL

Services now, platform licences later

- Advisory £1,200–£1,800/day
- Consulting sprints £15k–£60k
- Pilots £40k–£150k
- Per-woman-per-year licences (UK £8–£40; Africa £3–£12 at scale)
- E-learning B2B from year 2 (all ESTIMATE)

Visual: Revenue-mix bar for the Base scenario by year.



What we have · what we are building · what is next

- Have: full company build, prototype under audit, prior ministry conversation in Nigeria
- Building: entity, IP deed, advisory board, research report, first engagements
- Next: first paid engagement (Day 45), one UK evaluation site and one Nigeria NGO cohort in design (Day 90)

Visual: Three-column card layout with dates.



COMPETITIVE LANDSCAPE

Record systems, consumer apps and donor programmes leave the coordination layer open

- Clinical records (BadgerNet, K2, Euroking)
- consumer/pregnancy apps
- donor-funded programmes (mDoc in Nigeria, MomConnect in South Africa)
- consultancies for the services pillar
- Vytalix's buildable advantages: evidence-first posture, interoperability, clinician governance, UK-plus-Africa design

Visual: 2x2: clinical depth vs reach across settings.



TECHNOLOGY

Lean, standard, secure

- Web/PWA front end
- API and PostgreSQL
- UK-region hosting with Nigeria residency options
- messaging gateways (SMS, WhatsApp)
- FHIR-aligned integration roadmap
- Cyber Essentials and DSPT controls
- no patient data to third-party AI without a DPIA

Visual: Architecture diagram from `/docs/07\_TECHNOLOGY\_ARCHITECTURE.md`.



ROADMAP

Eight quarters, gated by evidence

- Q4 2026 foundations and services
- 2027 MVP, service evaluation, NGO cohort
- 2028 formal evaluation, seed, second market
- 2029 scale decisions

Visual: Timeline with gates (entity, IP, pilot LOI, evaluation, funding).



TEAM AND GOVERNANCE

Founder-led, with the advisory board being recruited now

- Founder [name, background]
- Advisory board targets: obstetric/midwifery lead, NHS digital leader, Nigerian health-system leader, regulatory/QA adviser, investor
- First hires funded by the round: product lead, fractional clinical safety officer

Visual: Org chart with "recruiting" markers; no invented people.



Three scenarios, one model

- Base three-year revenue £1.47m; services 59%
- Peak cash need £229k (Conservative) to £1.06m (Base)
- Break-even not reached in 36 months in Base; sustained EBITDA depends on retainers and pilots converting
- Full model: `/finance/Vytalix\_Financial\_Model\_3yr.xlsx`

Visual: Annual revenue and EBITDA bars for three scenarios.



THE RAISE AND THE ASK

Pre-seed £250k–£500k to build the MVP and run two pilots

- Use of funds: product lead, clinical-safety artefacts, MVP build, two pilots, compliance
- SEIS/EIS advance assurance applied for after incorporation (TO VALIDATE)
- Milestones before close: entity, IP deed, first revenue, pilot LOI
- Later rounds only on evidence

Visual: Use-of-funds donut; milestone list.

